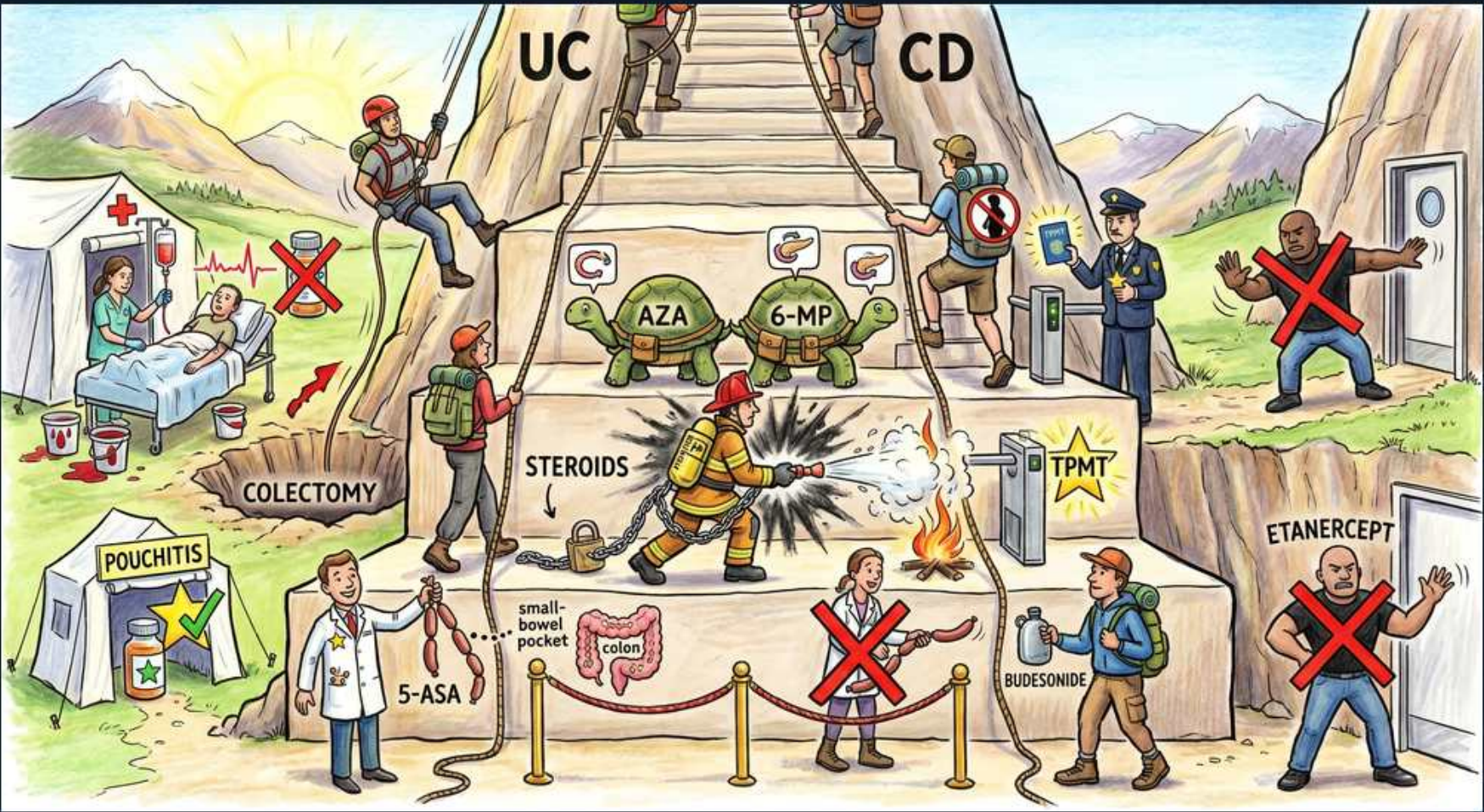


IBD — Treatment Ladder



1. Staircase ladder UC vs CD

WHAT YOU SEE

Central staircase splitting into 'UC' (left) and 'CD' (right) paths.

WHAT IT ENCODES

Step-up therapy: mild → 5-ASA/budesonide; flares → steroids (induction only); maintenance → AZA/6-MP, MTX; refractory/severe → biologics, cyclosporine rescue, surgery. The two paths emphasize UC-specific vs Crohn-specific agents.

2. 5-ASA pharmacist (gold star)

WHAT YOU SEE

Pharmacist at the base of the UC side labeled '5-ASA'.

WHAT IT ENCODES

5-ASA (mesalamine/sulfasalazine) is the gold-standard MAINSTAY for UC — both induction and maintenance of mild–moderate disease — acting topically on colonic mucosa. Gold star = first-line for UC.

3. 5-ASA crossed on CD path (red X)

WHAT YOU SEE

A 'small-bowel pocket / colon' figure on the CD steps blocked by a velvet rope and slashed with a red X — 5-ASA barred from the Crohn's path = ineffective for CD maintenance.

WHAT IT ENCODES

5-ASA is NOT effective for Crohn's maintenance — it does not reliably reach transmural small-bowel disease and lacks maintenance benefit in CD. A classic trap: do not use 5-ASA to maintain Crohn's remission.

BOARD TRAP

"5-ASA maintains Crohn's remission" → wrong; 5-ASA is ineffective in Crohn's, especially small-bowel disease.

4. Budesonide hiker

WHAT YOU SEE

Hiker on the CD side labeled 'BUDESONIDE'.

WHAT IT ENCODES

Ileal-release budesonide is used for mild–moderate ILEAL/right-colon Crohn's. High first-pass hepatic metabolism limits systemic steroid side effects. Induction agent, not long-term maintenance.

5. Steroids firefighter

WHAT YOU SEE

Firefighter labeled 'STERIODS' spraying out a fire mid-staircase.

WHAT IT ENCODES

Systemic corticosteroids INDUCE remission in flares of UC or Crohn's but are NEVER for maintenance (no mucosal healing, cumulative toxicity: osteoporosis, hyperglycemia, infection). Always taper and transition to a steroid-sparing agent.

6. AZA & 6-MP turtles

WHAT YOU SEE

Two turtles labeled 'AZA' and '6-MP' on the upper steps.

WHAT IT ENCODES

Azathioprine / 6-mercaptopurine are slow-onset (weeks–months, the turtles) STEROID-SPARING maintenance agents. Their pancreatitis is reversible on stopping; monitor for myelosuppression and hepatotoxicity.

7. TPMT star

WHAT YOU SEE

Yellow burst labeled 'TPMT' near the right steps.

WHAT IT ENCODES

Check TPMT (thiopurine methyltransferase) activity BEFORE starting AZA/6-MP — low/absent TPMT → toxic metabolite accumulation → severe myelosuppression. Low TPMT requires dose reduction or avoidance.

8. Cyclosporine rescue climber / colectomy tent

WHAT YOU SEE

Climber up the left cliff above an IV/hospital tent with red-X crossed.

WHAT IT ENCODES

Severe steroid-refractory UC: IV cyclosporine (or infliximab) is RESCUE therapy to avoid emergency colectomy. Severe UC flare → IV steroids first; if no response in ~3–5 days, rescue with cyclosporine/infliximab.

9. Pouchitis antibiotics (gold star)

WHAT YOU SEE

Tent labeled 'POUCHITIS' with a gold star and pill bottle.

WHAT IT ENCODES

Antibiotics (ciprofloxacin/metronidazole) are first-line for POUCHITIS after ileal pouch–anal anastomosis in UC. Gold star = correct antibiotic indication in IBD.

10. Antibiotics crossed for active UC (red X)

WHAT YOU SEE

A firefighter-style figure mid-staircase aiming a hose at the UC flare, slashed with a red X — antibiotics aimed at active UC = the wrong tool.

WHAT IT ENCODES

Antibiotics do NOT treat active UC colitis — they are reserved for pouchitis or perianal Crohn's/abscess. Using antibiotics to treat an active UC flare is a trap.

BOARD TRAP

"Antibiotics treat active ulcerative colitis" → wrong; antibiotics are for pouchitis, not active UC.

11. Etanercept bouncer (red X)

WHAT YOU SEE

Bouncer at a doorway crossed with a large red X, labeled 'ETANERCEPT'.

WHAT IT ENCODES

Etanercept (soluble TNF-receptor decoy) does NOT work in IBD — it is effective only for joint disease (RA, AS, psoriatic arthritis). Effective IBD anti-TNFs are the monoclonals infliximab and adalimumab.

BOARD TRAP

"Etanercept is an effective IBD biologic" → wrong; it works for joints only, not IBD; use infliximab/adalimumab.
